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The particularity of freedom and recovery from addiction in practice: On the therapeutic limitations of generalising addiction science

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ABSTRACT

Addiction science is very often cast as an indispensable source of political justification and clinical resources for engaging addicts therapeutically rather than punitively. It is said to promote an image of overcoming addictions as projects of recovery from disease or mental disorder rather than merely a repudiation of one's formerly troubled ways. Thusly, it is said to inform more compassionate and effective approaches to fostering constructive personal change than merely blaming and punishing addicts for their difficulties. In this essay I argue that because mainstream addiction science seeks to generalise regarding the nature of addiction and recovery, it invariably fails to capture the practices of freedom through which particular people undertake to recover from their particular addictions. I show how theoretical resources drawn from Michel Foucault, Harry Frankfurt and Donald Davidson allow us to develop the insights of posthumanist scholarship to provide in detail for the specific practices of freedom through which people undertake to recover from their particular addictions under the diverse practical conditions in which they seek to do so. This attention to particularity keys addiction science much more closely to the actual exigencies people encounter in the work of therapeutically fostering recovery from addiction in practice.

In 2006, the Betty Ford Institute, one of the leading addiction treatment providers in the United States, convened what they called a "consensus panel" of twelve established figures in the addiction treatment, policy and research fields to begin to produce a working consensus on the definition of recovery. They noted the ubiquity of this term in the field but a striking lack of consensus, and even concern, as to what this term actually means (The Betty Ford Institute Consensus Panel, 2007). They suggested that without consensus, the work of comparing treatment outcomes across various modalities of intervention; fostering clear communication both within addiction services and between those services and the wider public; and developing adequately scientific (that is, generalisable) measures of recovery are all very seriously encumbered (see also White, 2007). I don't dispute this.

However, in this essay I argue that this emphasis on definitional consensus, and its implicit valorisation of scientific generalisability, produces encumbrances of its own. More specifically, if we agree with most addiction experts that the core criterion of addiction is the loss of self-control, we must note that the valorisation of scientific generalisability regarding recovery entails thinking about the objective of recovery–the resumption of self-control, or the addict's return to personal freedom–in such a way that one general definition of freedom will suffice in all cases. For reasons I explore in what follows I am not convinced that, in the specifically therapeutic context, this is a productive way forward. In liberal and multicultural societies, freedom means many different things to different people. Hence, fostering people's freedom from the putative slavery of their addictions must entail a fundamental appreciation of the particularity of their own aspirations, opportunities, aptitudes and desires. In place of a concern for definitional consensus, I recommend a concern for what the philosopher Ludwig Wittgenstein called family resemblances.[1]

We can begin to address this later concern by noting the remarks made by the UK's Sainsbury Centre for Mental Health in their paper entitled "Making Recovery a Reality" (2008). Here they summarise some of the main emphases that any recovery oriented approach to mental health might wish to keep in mind:

[1] In his *Blue Book*, Wittgenstein (1958, p. 17) wrote of a pervasive "craving for generality" among both philosophers and scientists that, he argued, hailed from a number of sources, one of which was,

> *The tendency to look for something in common to all the entities which we commonly subsume under a general term…We are inclined to think that there must be something in common to all games, say, and that this common property is the justification for applying the general term "game" to the various games; whereas games form a family the members of which have family likenesses. Some of them have the same nose, others the same eyebrows and others again the same way of walking; and these likenesses overlap.*

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- Recovery is about building a meaningful and satisfying life, as defined by the person themselves, whether or not there are ongoing or recurring symptoms or problems.
- Recovery represents a movement away from pathology, illness and symptoms to health, strengths and wellness.
- Hope is central to recovery and can be enhanced by each person seeing how they can have more active control over their lives ('agency') and by seeing how others have found a way forward.
- Self management is encouraged and facilitated. The processes of self-management are similar, but what works may be very different for each individual. No 'one size fits all'.
- The helping relationship between clinicians and patients moves away from being expert/patient to being 'coaches' or 'partners' on a journey of discovery. Clinicians are there to be 'on tap, not on top'.
- People do not recover in isolation. Recovery is closely associated with social inclusion and being able to take on meaningful and satisfying social roles within local communities, rather than in segregated services.
- Recovery is about discovering or re-discovering a sense of personal identity, separate from illness or disability (quoted from Thornton & Lucas, 2011, p. 24)

As Thornton and Lucas (2011) observe, strictly speaking these principles do not comprise an integrated model or definition of recovery. Instead they are better understood as a set of only loosely related sensibilities. However, they do serve to highlight a few points that I believe ought to be at the centre of our reflections on the nature of addiction and recovery in practice. In the first instance, recovery is often best understood as patient-centred. That is, the nature of and path to recovery are, in liberal societies at least, often best understood as in the eye of the beholder. We are all, by these lights, entitled in the last analysis to decide for ourselves what it means to personally flourish or to overcome the afflictions by which we are presumed to suffer. In short, the recovery ethos is here cast as fundamentally at odds with the paternalistic ethos found in most mainstream medical, including addiction, science. Second, recovery is here seen to be less about the absence of pathology narrowly conceived and more about quality of life and belonging (Weinberg, 2005). And third, recovery is here understood to be about self-actualisation in the sense of self-discovery, self-empowerment and our practices of personal freedom in contrast to the alienation, bewilderment, disempowerment and un-freedom of addiction.

After briefly reviewing the therapeutic limitations inherent to the generalising impulses of mainstream addiction science, I show how Michel Foucault's thought on what he called practices of freedom furnishes some basic theoretical resources with which to capture the particularity of personal freedom and recovery from addiction. I also show how Foucault's thought can be usefully refined in this regard by recourse to Harry Frankfurt's ruminations on the practicalities of freewill (or distinguishing in practice those mental contents 'internal' to the self and self-control from those that are 'external,' harmful or threatening to particular selves). The argument is further refined with recourse to Donald Davidson's famous reflections on mental compartmentalisation, the principle of charity, and the difference between reasoned/willed action and behaviour caused by mental structures but which is nonetheless unreasoned/unwilled. Specifically how these particularist ideas can be made useful to empirical research on addiction and recovery is shown by means of a brief analysis of some ethnographic data collected in an addiction treatment programme and a more inclusive discussion of posthumanist scholarship on subjectivity, the body and addiction in practice.

A brief review of the state of the art in addiction science

Despite a range of withering critiques, what the historian David Courtwright (2010) has dubbed the NIDA (National Institute on Drug Abuse) brain disease paradigm continues to dominate international addiction science and clinical care (cf. Volkow, Koob, & McLellan, 2016). Champions of this paradigm have consistently emphasized the importance of defending a medical frame for drug problems not only because they believe the best research suggests the scientific truth of this frame but because they believe it is politically and culturally indispensable that we define addiction as a disease if it is to be met with enlightened societal responses, sympathy rather than scorn (cf. Leshner, 1997). How, then, is the case made for this medical frame? In a widely cited article in the *Annual Review of Neuroscience*, the distinguished neurologist Steven Hyman and his colleagues wrote,

> *Unlike natural rewards, addictive drugs do not serve any beneficial homoeostatic or reproductive purpose but instead often prove detrimental to health and functioning. Much work over several decades has begun to paint a picture of how addictive drugs come to masquerade as, and eventually supplant, natural rewards as highly valued goals.* (Hyman, Malenka, & Nestler, 2006, p. 571).

It is by way of this general distinction between natural and unnatural rewards that a scientific case is made for the argument that addictive drugs cause a loss of self-control, and precisely thereby, create a medical disability warranting medical care (cf. Kalivas & Volkow, 2005; Kelley & Berridge, 2002; Robinson & Berridge, 2003). By *unnaturally* 'hijacking' the brain's natural proclivities to reward healthy behaviour, addictive drugs usurp and enslave the brain's reward circuitry, rendering untold harms to victims themselves and to their societies (cf. Volkow et al., 2016, p. 364). Ironically, there is implicit in this wholly deterministic account of the brain's biomechanical responses to addictive chemicals a distinct (if dubious) general theory of freedom. If addiction, or the loss of self-control, flows from our sustained indulgence in the unnatural rewards of drug use, then recovery of our self-control, must, by implication, invariably consist largely in confining our attention to natural, or biologically healthy, rewards.

But the narrow equation of self-control with a general understanding of health maintenance is not scientifically sustainable. In modern liberal societies not all that is unhealthy is therefore unfree, let alone pathological. Beyond valorising health, addiction scientists must tell us how people actually grow estranged from their behaviour enough to warrant the claim their self-control is genuinely afflicted by addiction. Many have sought to do so by arguing addictions compromise the brain's 'executive functions' (cf. Kalivas & O'Brien, 2008). These functions include things like attention, response inhibition, planning, problem solving, and working memory. But again, in practice, it is simply false to say people universally equate self-control with long-term planning, problem-solving and impulse control.

Instead, we often equate self-control with following our gut instincts, spontaneous desires, and other emotional impulses and, indeed, equate the kinds of cognitive processes associated with executive function with alienation from our real selves and authentic self-control (cf. Alasuutari, 1992; Hochschild, 2012; Turner, 1976). A vivid and pertinent illustration of this is the extensive therapeutic emphasis recovery programmes like Alcoholics Anonymous place not on the executive repression of spontaneous emotions but encouraging their free and open expression in the interest of facilitating people's better self-understanding and self-control (Carr, 2011; Garriot & Raikhel 2015; Valverde, 1998; Weinberg, 2013). Because they fail to adequately explain the myriad realities of addiction and recovery in practice, NIDA brain disease boosters also ultimately fall short of fulfilling their self-proclaimed ethical objective of providing a general scientific warrant for therapeutic care.

The main alternative to the NIDA brain disease paradigm is the argument that addiction is invariably a free choice that does not compromise self-control (cf. Davies, 1992; Heather & Segal, 2017; Heyman, 2009; Levy, 2013; Lewis, 2018). Choice theorists often begin with the observation that most behaviour attributed to addictions is incentive sensitive (that is, at least partially consistent with cost-benefit analysis). While this is certainly correct, we should not make too much of it. Plants are incentive sensitive in the sense that they grow toward resources like

light and water but we would not normally want to conceptualize this as a voluntary choice.

Despite holding addiction is incentive sensitive, most choice theorists have abandoned the effort to explain addiction with orthodox rational choice theory. For example, George Ainslie (cf. 1992), and others, have influentially argued addicts are prone to 'hyperbolically discount' the prospects of future costs and benefits. In other words, the greater the expectation of immediate reward and the greater the expected intensity of that reward, the more we become tempted to abandon our longer term plans and preferences. Hyperbolic discounting, though, is not a form of deliberation confined to addicts. It is a widely shared tendency we all exhibit whether we are addicts or not. Hence, by closing the distance between addiction and generic features of human choice making, choice theorists tend to normalize addictive reasoning and volition. This makes it difficult to see why or how addiction should warrant suspension of our ethical accountability for our actions in favour of more therapeutic, sympathetic and exculpatory stances.

Therapeutic work not only requires theories that plausibly explain or justify addicted behaviour; it specifically requires theories that allow these behaviours to be, at least partially, ethically disowned. It is only by plausibly distinguishing addicts' free agency, their self-control, from the effects of their addictions, that people might be simultaneously understood as amenable to therapeutic empowerment and somehow also afflicted by an addiction that justifies and demands such a therapeutic engagement in the first place. Furthermore, those struggling to overcome addictions require theories that go beyond merely providing credible justification for absolution, or the disowning of past misdeeds. As all who have sought to care for addicts know, recovery is an ongoing exercise of learning to anticipate and prevent the rekindling of people's addictions, and it requires theories that not only facilitate, but actually warrant, such projects in the first place (Weinberg, 2000). Choice theorists have made little, if any, headway in this regard.

In sum, addiction science and policy remain to this day largely torn between the Scylla of a generalised biomechanical reductionism and the Charybdis of a generalized liberal voluntarism. In both cases, addiction is invariably cast simply as an extreme and harmful form of lust. And in both cases, little, if any, scientific warrant is provided for distinguishing the self-governed pursuit of happiness from the personally destructive loss of self-control. Because addicts' loss of self-control is so widely presumed to be the sole warrant for a therapeutic rather than punitive orientation to intervention, this is a rather consequential limitation.

On practices of freedom: Foucault, Frankfurt and Davidson

Contrary to both generalised biomechanical approaches and the generalised and reified voluntarism of the generic liberal subject, Michel Foucault insisted freedom and ethics take their characteristic forms as diverse, contextually embedded, products of the specific constellations of power relations within which they emerge and are sustained. Opposing the apostles of unfreedom, Foucault (cf. 1987, p. 114) insisted we take seriously the workings of personal responsibility and discretion exercised through what he sometimes called "practices of freedom." To those who posit a generic self-governing, largely rational, liberal subject as an axiomatic historical constant, Foucault insisted the reality of freedom cannot be adequately specified either in opposition to determinism or without reference to the historically and culturally specific horizons of possibility articulated in and through power relations. In an important further development of this insight, Nikolas Rose, for example, has long been at the forefront of a group of scholars focused on the Foucauldian concept of 'governmentality,' or the conduct of conduct, by which various sorts of power/knowledge regimes are given expression not through violence or coercion but through the very operations of freedom itself (cf. Burchell, Gordon, & Miller, 1991; Reith, 2004; Rose, 1990, 2006).

This group has produced an impressive collection of studies that look at governmentality as it occurs through disciplinary programmes including, notably, what Rose has called the 'psy' disciplines, like psychiatry, but also a number of other administrative disciplines including economics, politics and indeed Alcoholics Anonymous. These studies persuasively demonstrate just how, through the powerful operation of liberal and neo-liberal institutions, people are variously "obliged to be free in certain ways" (Rose et al., 2006, p. 89) and obliged to conceive of their own freedom in the terms and techniques furnished by different disciplinary matrices. Liberal forms of freedom are shown to be actualised less through resistance to these regimes than through people's participation in them.

However, by so closely yoking their studies of specific practices of freedom to institutionalised forms of disciplinary expertise, these scholars make it rather difficult to understand what, if any, differences might be drawn between an acquiescence to domination and freedom itself. Moreover, they are unclear as to the ontological status of the media upon which disciplinary technologies are exercised and the ethical ramifications thereof. As Foucauldians, they can't easily argue these media consist of biological bodies as biology is also understood as a historically and culturally specific disciplinary technology itself. The same goes for psychological minds. But despite this ambiguity there nonetheless remains a consistent implication in this literature that whatever else these media may be they are always already human, self-governing and ethically accountable subjects. Though sometimes professing affinities between their own approach and what they call the 'antihumanist stance' of Actor Network Theory (cf. Rose et al., 2006, p. 93), their interest in non-human agency has been confined to such matters as prison cells and has not been applied to understanding the specifically intrapersonal forces that may occasionally vex our freedom. As will be shown, this glossing of that which is interior to us as persons but exterior to the free subjectivity of our selves[2] rather limits the potential of this research tradition in certain important ways.

This cannot be quite so easily said of Foucault himself. While insisting that "power exists only over free subjects, and only insofar as they are 'free'" (Foucault, 1983, p. 221), Foucault explicitly contrasted this form of effectuality from the kinds of control that are exercised over things—that is, unfree non-subjects (Foucault, 1983). Immediately following the passage just quoted, Foucault (1983, p. 221) goes on to write, "By this we mean individual or collective subjects who are faced with a field of possibilities in which several ways of behaving, several reactions, and diverse comportments may be realized. Where the determining factors saturate the whole there is no relationship of power; slavery is not a power relationship when man [sic] is in chains. (In this case it is a question of a physical relationship of constraint.)"[3] And most importantly for my argument here, Foucault wrote that while ethical practices are only possible for free subjects rather than slaves or non-subjects like prison cells or addictions, conversely, practices of freedom are intrinsically ethical practices.[4]

Of course, Foucault was well aware of the fact that establishing whether a relationship is one of power over free subjects or one of brute

[2] I am relying here on Rom Harre's (1987, p. 42) distinction between the *person* (understood as the 'human being as a social individual embodied and publicly identifiable') and the *self* (understood as 'that inner unity to which all personal experience [and I would add, conduct] belongs as attributes of a subject.').

[3] As will be further discussed below, I don't think brute determinism is confined to physical constraint.

[4] This is made explicit in the following interview exchange,

> Foucault: …is it obvious that in liberating one's desires one will know how to behave ethically in pleasurable relationships with others?
> [Interviewer]: You say that liberty must be practiced ethically?
> Foucault: Yes, for what is morality, if not the practice of liberty, the deliberate practice of liberty?
> [Interviewer]: That means that you consider liberty as a reality already ethical in itself?
> Foucault: Liberty is the ontological condition of ethics. But ethics is the deliberate form assumed by liberty. (Foucault, 1987, p. 115)

determinism often requires a subtle hermeneutics. But there can be no disputing that Foucault provided explicitly for an ethical exterior to the power dynamics that govern relationships between free subjects and those subjects with themselves. To my knowledge, though, he himself never explored this matter in any detail. More specifically, he did not provide for the practical dynamics that produce, nor the resulting ontological status of, those elements of our being, like addictions, that we dissociate from freewill or self-control. Given his deep appreciation for the historicity, contingency and lability of social life (or to use his own word, its microphysics), it is safe to assume Foucault would have been sensitive to the multiplicity of patterns by which boundaries are drawn between power dynamics and their ethical exteriors. But while Foucauldian insights are immensely conducive to highlighting and examining these patterns, they are not sufficient. We must more closely consider than did Foucault the processes through which particular people evaluate their own and one another's self-identification with, as well as alienation from, their thoughts, feelings and behaviours. In my view, the most fruitful path forward in this respect has been charted in the work of two important and influential philosophers, Harry Frankfurt and Donald Davidson.

In his essay "Freedom of the Will and the Concept of the Person," Frankfurt (1971) drew a distinction between what he called first and second order desires. He argued people are not only capable of forming preferences for one thing or another but we are also capable of taking these first order preferences as matters for evaluation and either self-identification or dissociation from our selves. We can want things on the first level that we wish we didn't want on the second level and, likewise, we can not want things on the first level we wish we did want on the second. Moreover, we can experience both first and second order desires as 'internal' and expressive of our freewill and character as unique selves, as 'external' and sources of constraint or affliction or as matters about which we are ambivalent. Freewill is not, on Frankfurt's account, exhibited in the mere fact of unfettered desire but is to be found in what he ultimately called our satisfaction with our desires. Rather helpfully to my own cause, he explicitly used the notion of addiction to make his case.[5]

This essay kick started fairly extensive debates over its merits and over the decades since its publication Frankfurt himself was occasionally drawn to respond. Perhaps the most important critique of his original account is that it results in an infinite regress. Critics have noted that in that account Frankfurt gives no reason to presume that our meta-reflections regarding which of our desires are and are not self-expressive conclude with the second order. To quote Frankfurt (1971, p. 16),

> *There is no theoretical limit to the length of the series of desires of higher and higher orders; nothing except common sense and, perhaps, a saving fatigue prevents an individual from obsessively refusing to identify himself [sic] with any of his desires until he forms a desire of the next higher order. The tendency to generate such a series of acts of forming desires, which would be a case of humanization run wild, also leads toward the destruction of a person. It is possible, however, to terminate such a series of acts without cutting it off arbitrarily. When a person identifies himself decisively with one of his first-order desires, this commitment "resounds" throughout the potentially endless array of higher orders.*

But critics noted that if to conclude regress the actor must actively endorse or identify "decisively" with a first order desire without thereby implicating a reason– inherently vulnerable to still further doubt–then it would appear that this conclusion must in fact be reasonless, or arbitrary. In his essay "The Faintest Passion," Frankfurt (1999) amended his position to suggest that we self-identify with an element of our mental life when we are "wholehearted" with respect to it. He defined this as being "satisfied" with that element which he further specified as follows,

> *To be satisfied with something does not require that a person have any particular belief about it, nor any particular feeling or attitude or intention. It does not require, for instance, that he [sic] regard it as satisfactory, or that he accede to it with approval, or that he intend to leave it as it stands. There is nothing that he needs to think, or to adopt, or to accept; it is not necessary for him to do anything at all…Satisfaction with one's self requires, then, no adoption of any cognitive, attitudinal, affective, or intentional stance. It does not require the performance of a particular act; and it also does not require any deliberate abstention. Satisfaction is a state of the entire psychic system—a state constituted just by the absence of any tendency or inclination to alter its condition.* (Frankfurt, 1999, p. 104).

Frankfurt (1999, p. 105) further argued, "satisfaction with one psychic element does not require satisfaction with any other," so his is emphatically not a theory that requires a rational coherence or consistency amongst all those psychic elements with which we self-identify. Moreover, he also noted "the extent to which people suffer from volitional indeterminacy is not entirely independent of the social, political, and cultural conditions in which they live. Those conditions may either facilitate or impede the development of unambivalent attitudes, preferences, and goals" (Frankfurt, 1999, p. 102). So what, then, might Frankfurt have meant when he argued that despite its not requiring rational coherence or the conscious, let alone deliberate, commission or omission of any active performance, that satisfaction must nonetheless be "a state of the entire psychic system"? Or that, "insofar as his desires are entirely unreflective, he is to that extent not a person at all. He is a wanton."? (Frankfurt, 1999, pp. 105–6) This puzzle is best solved by clarifying what Frankfurt meant by "unreflective." He meant by this not inactive or unconscious, both of which self-identification may very well be, but rather, reason insensitive. That is, in order to retain one's status as a person (as opposed to a mere wanton) one must remain to be seen to be amenable, at least in principle, to reasonable persuasion with respect to the psychic elements with which one self identifies or, for that matter, with which one does not. One need not actually put these in question but one must, at least, be amenable to their reasonable interrogation.[6]

This argument suggests in the first instance that self-identification, or distinguishing those psychic elements Frankfurt called 'internal' to the self from those he called 'external'—those with which we self-identify and from which flow our own self-willed actions from those, like addictions, we find alien and from which flow constraints on, or afflictions of, our freedom—will invariably be a matter of particular selves and the particular ethical and cultural contexts (or in Foucault's terms, the particular microphysics of power) in which they find themselves in all of their specificity. Secondly, one's self is here identified as those elements of our psychic life that are held expressive of our particular character whether they are emotionally repressive in the name of our long term interests and preferences or emotionally expressive of our natural and often spontaneous inclinations including our inclinations toward the people or things we love or which give us pleasure. While Frankfurt (1999) insisted we must be wholehearted in our identifica-

[5] I should be clear that I find Frankfurt's analysis of what he called the "willing addict" unhelpful and as entailing the embrace of an oxymoron. By my own account one may willingly indulge self-destructive habits of one kind or another. But the expression willing addict is to my mind equivalent to the expression free slave and, therefore, incoherent.

[6] On this point Frankfurt (1999, pp. 104-5) wrote,

> *Being genuinely satisfied is not a matter, then, of choosing to leave things as they are or of making some judgment or decision concerning the desirability of change. It is a matter of simply having no interest in making changes. What it requires is that psychic elements of certain kinds do not occur. But while the absence of such elements does not require either deliberate action or deliberate restraint, their absence must nonetheless be reflective. In other words, the fact that the person is not moved to change things must derive from his [sic] understanding and evaluation of how things are with him. Thus, the essential non-occurrence is neither deliberately contrived nor wantonly unselfconscious. It develops and prevails as an unmanaged consequence of the person's appreciation of his psychic condition.*

tion of those elements internal to our selves, this is not necessarily a matter of what philosophers sometimes call 'all things considered' rational evaluations but may also include, much more modestly, efforts to merely minimise ambivalence. Ambivalence only arises when we actually become aware of conflicts between elements of our psychic life.

There is a ubiquitous tendency in addiction science to assume the existence of a general 'ideal rationality,' the normative principles of which hold universally—that is, regardless of social context. By these lights, rationality and, hence, self-control invariably consists in honouring or being guided by these principles. Whenever or wherever someone seems not to be guided by them we are presented with the challenge of explaining their evident irrationality and loss of self-control. While there remain lively controversies in the philosophical, psychological and economic literatures pertaining to the specific identity of these principles, the details of these debates need not concern us here. Suffice to say these debates are largely about the causes, characteristics and consequences of this ideal rationality and much less concerned with whether we ought to think of it as universal. The principles of rationality I have in mind include but are not limited to the normative requirements that our beliefs, intentions and actions be coherent, consistent and stable.

It is often said that Donald Davidson himself held this view of rationality and that it undergirds his famous arguments concerning mental holism, the principle of charity and, in turn, via the notion of 'all things considered' judgments, akrasia or weakness of the will. There is certainly textual evidence for the truth of this claim. However, I think it is in need of important qualification. In his essay "Paradoxes of Irrationality," Davidson (1982) went beyond his earlier descriptive account of akrasia to develop a prospective explanation for how it could come about. He wrote,

> *Mental phenomena may cause other mental phenomena without being reasons for them, then, and still keep their character as mental, provided cause and effect are adequately segregated…To constitute a structure of the required sort, a part of the mind must show a larger degree of consistency or rationality than is attributed to the whole…The idea is that if parts of the mind are to some degree independent, we can understand how they are able to harbor inconsistencies, and to interact on a causal level…Recall the analysis of akrasia…the way could be cleared for explanation if we were to suppose two semi-autonomous departments of the mind, one that finds a certain course of action to be, all things considered, best, and another that prompts another course of action. On each side, the side of sober judgement and the side of incontinent intent and action, there is a supporting structure of reasons, of interlocking beliefs, expectations, assumptions, attitudes and desires.* (Davidson, 1982, p. 300).

In this essay, presumably because its central task was to engage with Freudian thought and, more specifically, to explain the possibility of irrationality, Davidson seemed to focus on formulating this partitioning of the mind as a segregation of its rational and irrational components (though he explicitly distinguishes this division from that between Freud's conscious and unconscious mind). If indeed his intention was confined to partitioning a singular "sober" agent from an "incontinent" one, we might very well want to insist that with his philosophy of mental holism, Davidson invariably identified self-control with that mental structure exhibiting the greatest conformity with a universal concept of ideal rationality. However, in a footnote he provided the following further specification of his position,

> *Here as elsewhere my highly abstract account of the partitioning of the mind deviates from Freud's. In particular, I have nothing to say about the number or nature of divisions of the mind, their permanence or aetiology. I am solely concerned to defend the idea of mental compartmentalization, and to argue that it is necessary if we are to explain a common form of irrationality. I should perhaps emphasize that phrases like 'partition of the mind', 'part of the mind', 'segment' etc. are misleading if they suggest that what belongs to one division of the mind cannot belong to another. The picture I want is of overlapping territories.* (Davidson, 1982, p. 300).

The facts that Davidson appeared agnostic as to the "number or nature of divisions of the mind," that he cast compartmentalisation as necessary to explaining a common form of irrationality but not obviously *confined* to the explanation of irrationality, and that these territories need not be mutually exclusive but are instead "overlapping," combine to suggest Davidson was at least open to an occasional degree of vagueness, equivocality and/or instability in our judgments as to which territory was best understood as the locus of self-control in any given case. In his highly Davidsonian discussion of Freud and moral reflection, Richard Rorty (1991, p. 151) quoted Philip Rieff's reference to "Freud's egalitarian revision of the traditional idea of a hierarchical human nature" wherein self-control is invariably identified with our 'higher' rational nature and set against our passions. Rorty argued Freud "gave us a vocabulary that lets us describe all the various parts of the soul, conscious and unconscious alike,…as equally plausible candidates for 'the true self'." (ibid., p. 152) and went on to write,

> *I want to focus on the way in which Freud, by helping us see ourselves as centerless, as random assemblages of contingent and idiosyncratic needs, rather than as more or less adequate exemplifications of a common human essence, opened up new possibilities for the aesthetic life. He helped us become increasingly ironic, playful, free, and inventive in our choice of self-descriptions. (ibid., p. 155).*

Rorty's attention to the rich variety of possibilities we enjoy in our particular choice of self-descriptions allows us to better appreciate the implications of Davidson's views on mental compartmentalisation. By Rorty's reading, Davidson probably didn't (and would have been wrong to) believe that rational requirements like coherence, consistency and stability were universal criteria with which to identify selves and self-control generically. It should also be recalled here that Davidson's principle of charity was plainly not a principle he thought was applied exhaustively and acontextually, for its own sake, to all of the mental contents with which people self-identify.[7] Instead its application was pragmatically driven in the first instance by an applicant's desire to get on with their lives in ways they find satisfying in the Frankfurtian sense and in ways that allow them to cooperate with others in their lives with whom they feel they must, should or would like to cooperate. The coherence we find in our own and each other's minds, then, tends to be anchored contextually—that is, in the particular forms of life within which we participate and which variously (and, to a considerable extent, discontinuously) make salient particular beliefs, intentions, values, and the practical dispositions or competences upon which these are grounded.

When we speak of holism in this context, then, it is crucial that we recognise we are usually referring to a holism bounded by inductive considerations of what might conceivably be relevant to particular, collectively orchestrated practical projects and decidedly not the much more inclusive holism suggested by many in both moral philosophy and addiction science—that is, a universal, wholly decontextualized, rule-based and consciously deliberated holism. By these lights, the partitions Davidson wrote of insulating various regions of our mental activity should not be understood as confined to partitions between the rational and irrational. Instead, they also extend to the porous boundaries we tacitly experience between the ecological contexts within which we acquire and incorporate our own particular region specific repertoires of dispositional competences. In short, whether or not the relations between elements of our mental life figure as reasons or as unreasoned causes of what we do is not only a matter of distinguishing rational from irrational motivations but of distinguishing context specific rationalities as well.

This can be seen in the following ethnographic fieldnote excerpt taken in a staff meeting of an out-patient recovery programme mandated to serve homeless and precariously housed clients suffering from

[7] Briefly, the principle of charity requires that we interpret our own and one another's thoughts and behaviours so as to maximize their rationality.

addiction and a concurrent major mental illness. In this case, a client, Layla,[8] had dropped out of the programme and was accumulating insurmountable debts using crack cocaine. She was contacted by a counsellor but resisted this counselor's efforts to place her into an inpatient recovery setting. In the absence of mitigating factors, counselors normally regarded such resistance as irrational and evidence, more specifically, of the insurmountable hold clients' addictions had over them. However, in this case there were mitigating circumstances and, despite the putative severity of Layla's relapse, her resistance was interpreted as a rational choice rather than a causal effect of her crack addiction:

> *She's resisting it because she doesn't want to loose her apartment. You know she moved into that apartment next to her parents' place, and it's kind of a good situation for her. If she goes into inpatient treatment she's afraid she'll loose her place. I can understand how she feels. She's in a tough situation.*

In this excerpt, knowledge of Layla's particular practical circumstances beyond the context of the programme led a counsellor to rationalize her resistance to treatment as strategically motivated by practical circumstances, rather than as evidence of the causal force of her addiction. However, more commonly, the immediate practical demands of participating in a recovery programme overrode whatever judgments might be based on practical circumstances clients faced outside the programme. Thus, a counsellor in the same programme elaborated on her difficulty getting Del to improve his personal hygiene and Neal to obtain false teeth:

> *People may be coming regularly, but if they continue to look like they're homeless and don't really care about their appearance, I can't help thinking they aren't bringing a lot of what they get in the programme out with them when they leave. I'm working with Neal to get his teeth again for the same reason. He looks awful without his teeth. I don't think looking like that can do anything for his self-image and self-esteem.*

Del, though, claimed to regard his lax personal hygiene not as evidence of his addiction but as an exercise in personal competence and a survival tactic while living homeless, mentioning to me later that he was more likely to be robbed if he looked better. Neal said essentially the same thing; that he fit into the homeless street scene better without his dentures and this kept him out of trouble. We can see in these examples the particularity, contingency, fluidity and, indeed, contestation that is often evident in determinations of whether particular people's actions are reasoned and self-governed or comparatively unreasoned and, hence, quite possibly evidence of the causal force of their addictions. In Layla's case, we might want to say Davidson's principle of charity was very obviously at work. However, the cases of Del and Neil are more complicated. While their counsellor was evidently devoted to helping them to elevate their self-image, self-esteem and, by implication, recovery of their self-control; they themselves felt this effort to empower them, guided by a well known standard of recovery culture (foster clients' self respect), could not be reconciled with the tactics they considered necessary to managing life on the streets. Here the question of which rationality should be seen to be the true locus of their self-control was not nearly so clear. There seems to have been a clash between the standards of rationality held characteristic of the recovery programme and the standards Del and Neal felt were most conducive to remaining safe on the streets. This clash served to vex interpretive judgments as to whether they were exercising a street-smart tactical self-control or succumbing to the irrational causal force of their addictions.

Posthumanism, recovery and the ontology of addiction in practice

Over the last two decades or so posthumanist scholarship has blossomed throughout the social sciences and, more specifically, in drug and alcohol studies (cf. Brookfield, Selvey, Maher, & Fitzgerald, 2021; Dilkes-Frayne & Duff 2017; Duff, 2018; Fraser, Moore, & Keane, 2014; Garriot & Raikhel 2015; Weinberg, 1997, 2005, 2013). Due precisely to this success, posthumanism is a concept that has come to mean many different things to different people. Because these meanings are often either incompatible or mutually irrelevant, the concept has sometimes appeared to both critics and champions alike as unwieldy or vague. In the interest of clarity, let me say that my own use of the concept is not intended as a blanket disavowal of humanist values like freedom, democracy and human rights. Nor is it intended to portend a future dominated by nonhumans of one kind or another. It is much more narrowly intended to express a disavowal of specific but ubiquitous conceptual generalisations regarding human nature, human action and/or the nature of the human condition. Most fundamentally, I am critical of the idea often found in the addiction science literature that freedom is a universal characteristic of human social behaviour only ever compromised by brute physical determinism. Secondly, I am critical of the ubiquitous tendency in the addiction literature to separate the mind, as subjective and sentient ethically accountable agent, from the body as objective biomechanical system.

Posthumanism provides resources with which to appreciate the particular practical contingencies of freedom and unfreedom in specific cases, the psychosomatic unity of the mind and body in practice, and to conceptually clarify and systematically investigate how it could be, and in fact is, that in practice particular people move into and out of self-control. It therefore provides a supple and dynamic orientation to studying what I have called the diverse ethical exteriors to Foucauldian practices of freedom—or the power relations between human subjects–and, hence, a means by which to warrant the therapeutic suspension of people's ethical accountabilities in precisely the ways I have shown both brain disease and choice theoretic orientations to addiction have hitherto failed to do so. The fundamental advantage of posthumanism is for me its general refusal to endorse any form of universal and a priori metaphysics concerning humanity, freedom or ethics and to instead study these ethnographically in what Harold Garfinkel called the perspicuous settings within which they take particular empirical form as the work products and objects of particular cohorts. However, I have also benefitted a great deal from the insights of specific scholars—most notably, Bruno Latour's (2004) insights respecting psychosomatic subjects and Annemarie Mol's (2002) insights concerning the multiplicity of the body.

Here is not the place to rehearse the diverse critiques that have been lodged against Cartesian dualism. From Marx, to American pragmatism, to Heidegger, Merleau-Ponty and existential phenomenology, a growing collection of researchers have argued respectively for an embodied understanding of our minds' practical activities and mindful understanding of our bodies' practical activities. In a particularly incisive study of the 'odour kits' perfume makers use to train 'noses' to detect progressively subtler aromatic contrasts, Latour (2004) makes a strong empirical case for such a psychosomatic subjectivity. In this study Latour demonstrates the body is not just an objective biomechanical system about which we can subjectively learn but, fundamentally, a mutable medium literally 'articulated' through its 'learn[ing] to be affected'. Latour goes a considerable distance towards creating a conceptual space within which addiction might be construed as an acquired but nonetheless eminently embodied form of subjectivity. By retaining a focus on the subjective, or lived sentient, body he accommodates much of what choice theorists seek to achieve by narrowing the distance between addiction and normal learning processes. But although he speaks of the multiplicity of environments within which the body subjectively learns to be affected, Latour does not address the multiplicity of the body itself—as not just an articulated medium for learning to be affected but a linked set of media for doing so. This limits his sensitivity to the potentials for embodied conflicts and/or afflictions like addiction.

Whereas Latour (2004) appears to suggest a normative judgment that any way that bodies learn to be affected by the world should be em-

[8] All names are pseudonyms.

braced, I want to be a bit more cautious. It seems to me that to have a body is also to be vulnerable to disease (cf. Mol, 2002; Mol & Law, 2004). However, as a posthumanist, I regard diseases not merely as pathological biological mechanisms, but, more generally, as patterns of harmful bodily articulation. That is, patterns of bodily articulation with which we in a Frankfurtian sense either cannot, or do not want to, identify as internal to our particular selves precisely because they afflict or endanger those articulations with which we do self-identify.

While we are all prone to greater and lesser degrees of psychosomatic integration, the failure of integration can sometimes become so extreme as to precipitate a radical disowning of particular psychosomatic subjectivities—a radical personal and ethical divestment from those subjectivities that effectively divorces certain constellations of entrenched habit from our own or our significant others' sense of who we really are—that is, the agency of our addictions from our free agency specifically as self-discovering, self-actualising and self-governing ethical subjects. Armed with such distinctions we are equipped to understand particular people, in all their specificity, as amenable to therapeutic empowerment or emancipation from their addictions but simultaneously afflicted by particular addictions that ethically justify and demand such a therapeutic engagement in the first place. We also become considerably more sensitive to the specific details of how addictions and recoveries come to be made manifest in practice across the myriad cases in which they are found to do so and thus better equipped to contribute to the recovery process in detail.

According to those who wish to contribute to a generalising addiction science, science must necessarily aspire to produce universally valid theoretical findings regarding such things as the self, self-control, the will, autonomy, freedom, addiction and, as we saw in the introduction, recovery itself. Whatever value addiction science may bring to the field of therapeutics is in this light cast uniformly in terms of the aptness of these universal models to the needs of caregivers and receivers. The thesis of this essay is that we might instead begin with research on the diverse and particular needs of caregivers and receivers in the myriad settings in which recovery is a concern. The insights so gleaned might then be used to: 1) compare (and perhaps explain) their family resemblances to one another, 2) interrogate the limitations of received universal models and, 3) thereby enrich our scientific understanding of addiction and recovery through a refinement and disaggregation of our knowledge of the various ways that these actually become manifest in practice.

Ethics approval

The authors declare that the work reported herein did not require ethics approval because it did not involve animal or human participation.

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